Assessment and Planning: high-yield review (about 32% of the exam)

Assess before you act

- New client, new symptom, surprising disclosure or change in functioning: **assess** (explore, screen, get collateral with consent, medical rule-out).
- Stem says the assessment is done, or states the findings: **move on** to planning or intervention. Don't re-survey.
- Asking about suicide is assessment, and it doesn't plant the idea. Emergency action needs intent, plan, means or imminence.

Risk quick list

- **Suicide:** ask directly about thoughts, intent, plan, means, past attempts, protective factors. Low risk: safety plan and limit means. Imminent: emergency evaluation.
- **Partner violence lethality:** past strangulation, gun access, recent or planned separation, escalating violence, threats to kill. Screen partners separately.
- **Child abuse:** injuries that don't fit the story, patterned marks, bruising in infants. Poverty alone is not neglect.
- **Elder abuse:** caregiver answers for the client, sudden money changes, unexplained injuries, untreated needs.

DSM thresholds (paraphrased)

Condition	Key detail
Brief psychotic / schizophreniform / schizophrenia	under 1 month / 1 to under 6 months / 6 months or more
Hypomania / mania	4 days, no marked impairment / 1 week, or hospitalized or psychotic
MDD / persistent depressive disorder	5 of 9 for 2 weeks / 2 years adults, 1 year youth
Acute stress / PTSD	3 days to 1 month / more than 1 month
Adjustment disorder	within 3 months of stressor; only if no other disorder fits
Prolonged grief	12 months after death (6 months youth)
GAD	6 months of worry plus symptoms
ADHD	6 months; several symptoms before 12; 2 or more settings
DMDD	outbursts 3+ a week plus irritability between; onset before 10
Major vs. mild NCD	major = needs help with complex daily tasks
SUD severity	2-3 mild, 4-5 moderate, 6+ severe

Rule out first: substance or medication, then medical condition, then a better-fitting disorder. Medical red flags: sudden onset, first episode after about 40, visual or tactile hallucinations, fluctuating attention, new medication, head injury.

Medication red flags (route to prescriber or emergency care; never adjust doses)

Picture	Think
Serotonergic combination; fast onset; clonus, fever, diarrhea	Serotonin syndrome: emergency
New antipsychotic; fever, rigid muscles, confusion	NMS: emergency
Lithium plus vomiting, coarse tremor, unsteadiness	Lithium toxicity: emergency
Restless, can't sit still after antipsychotic	Akathisia: tell prescriber
Abruptly stopping heavy alcohol or daily benzodiazepines	Seizure risk: medical withdrawal management
Antidepressant in a young person	Boxed warning: watch for suicidality

Development and defenses at a glance

- Erikson: trust, autonomy, initiative, industry, identity, intimacy, generativity, integrity.
- Piaget: sensorimotor (object permanence), preoperational (egocentrism, magical thinking), concrete (conservation), formal (abstract).
- Mahler rapprochement (about 15-24 months): clinging and pushing away. Disorganized attachment: linked to maltreatment.
- Defense pairs: suppression (conscious) vs. repression; displacement (new target) vs. projection (new owner); rationalization (excuses) vs. intellectualization (analysis); sublimation (mature) vs. displacement.

Tools and planning

- Screening is not diagnosis. PHQ-9 item 9 or EPDS item 10 positive: assess safety now.
- MSE: mood is reported, affect is observed; tangential never returns, circumstantial does.
- Genogram = generations; ecomap = current systems; culturagram = immigrant family context.
- Goals are SMART and the client's; choose the least restrictive level of care that is safe.
- Ready to end: goals met, client uses skills independently, gains hold between sessions.

Sources: DSM-5-TR (APA, 2022); Kaplan & Sadock's Synopsis (12th ed.); Stanley & Brown (2012); Campbell et al. (2009); ASAM Criteria (4th ed.); Erikson (1963); Piaget & Inhelder (1969); Mahler et al. (1975); Vaillant (1992).